National Mortality surveillance

Quarterly Bulletin

Issue: 1 **Issue Period: Jan—Jun 2025**

Executive Summary

The **global crude death rate** is estimated to be **7.7** deaths per 1,000 people each year while that for Sub-Saharan Africa stands at **8.0**[1]. Over the past decade, Uganda recorded a reduction in crude death rate from **7.7** in 2014 to **5.0** in 2024. Infant mortality reduced from **50** to **34** per 1,000 live births, while under-five mortality fell from **74** to **46** per 1,000 live births over the same period[2]. Despite this progress, both infant and under-five mortality remain above the **SDG 3.2** targets of **12 neonatal** deaths and **25 under-five** deaths per 1,000 live births respectively by **2030.**

This bulletin seeks to share mortality surveillance statistics with stakeholders, with an aim to **identify** deaths; **where**, **why**, and **when** they occurred; which age groups are most affected for purposes of informing public health surveillance and response, to inform evidence based decision making.

[1]The World Factbook (2022 Archive). [2]Uganda Bureau of Statistics. National Population and Housing Census - final report. 2024.

HIGHLIGHTS

In Uganda, mortality surveillance is implemented at both community and health facility levels. Health facility death data is captured both at the outpatient (HMIS 105), and Inpatient departments (HMIS 108) on a monthly basis. And on a quarterly basis, HIV/TB death data is largely reported through the HMIS 106a. This release is focused on analyzing death data from these 3 datasets.

Total Deaths 32496

OPD deaths	IPD deaths
26.9%	**73.1%**
(*n*=8744)	(*n*=23752)

Notified deaths

40.6% (*n*=13206)

Certified deaths

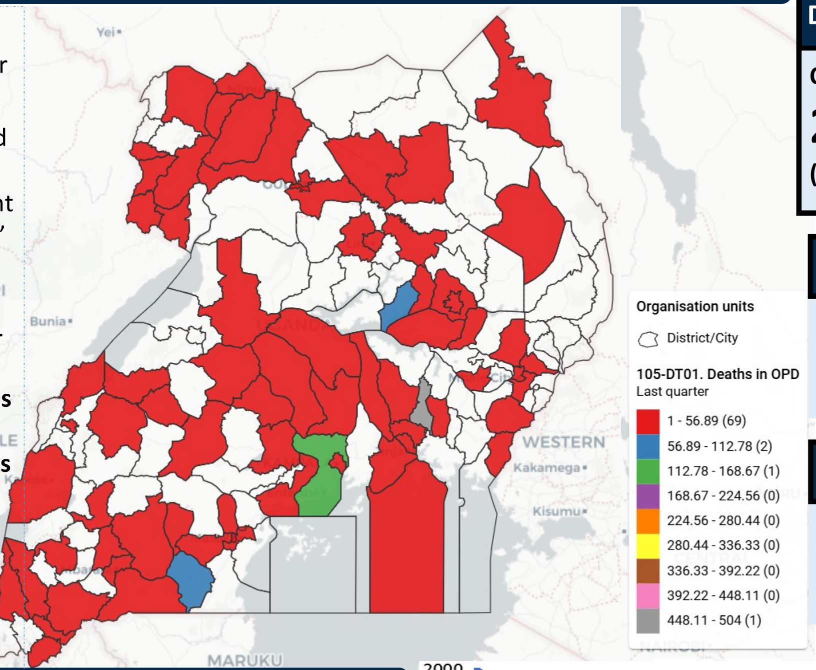
29.5% (*n*=9592)

Deaths at Outpatient

Deaths in OPD are reported under four categories:

1. Deaths recorded directly under the data element **"deaths in OPD"** *as seen in the Map*
2. **Deaths in Emergency Unit**
3. **Newborn deaths (0-7 days)**
4. **Maternal deaths**

For consistency, a **SUM** of the above should be used as total OPD deaths.



Deaths at OPD by Sex and Age Gp. (Jan-June)

Male	Age group	Female
1366	20+Yrs	1472
279	10-19Yrs	371
151	5-9Yrs	203
291	29days-4Yrs	301
401	0-28days	31

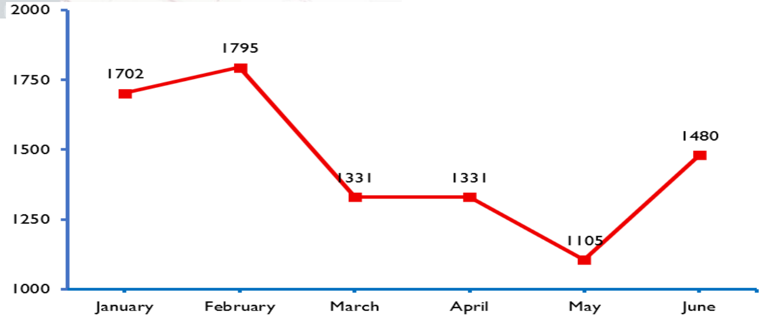
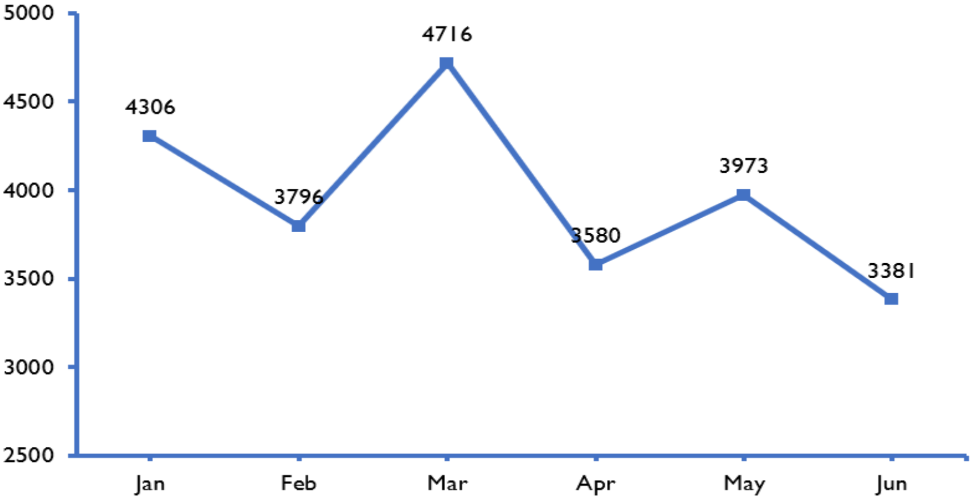


Figure 1: OPD mortality data shows a declining trend between February and May and then a rise in June

Deaths at Inpatient

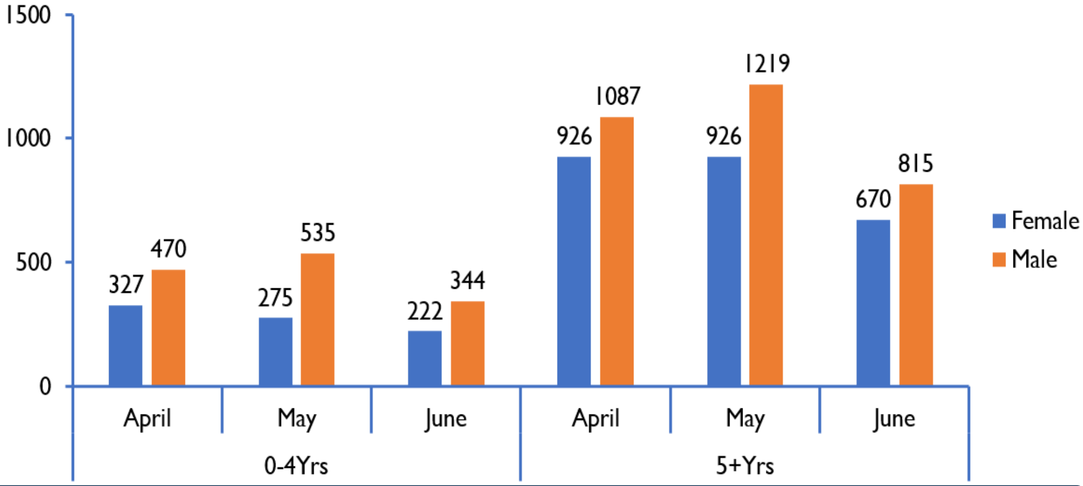


In the Inpatient Department (IPD) monthly report, total deaths are reported under the data element "**Total deaths**", however, the "(calculated)" total deaths at IPD was obtained by aggregating all individual variables with deaths.

It is therefore important to have the aggregated total IPD deaths **auto-calculated in**

Figure 2: Shows a trend of Deaths at IPD from Jan to June 2025.

Figure 3 (Right): At IPD, we generally loose more males compared to females across the reviewed months with the **"5+yrs"** age category being the most affected.

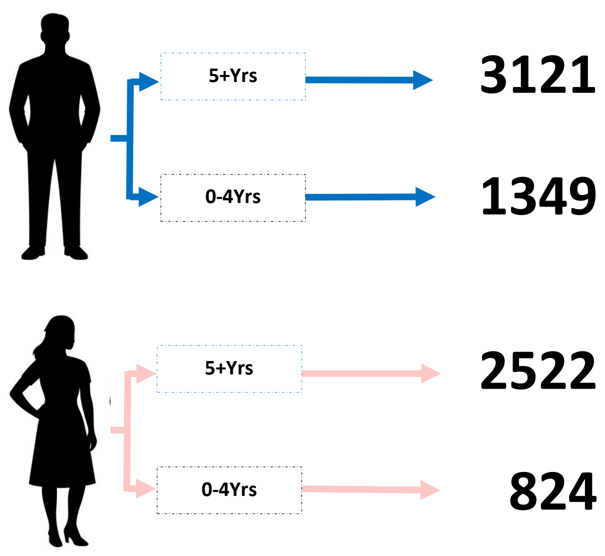


Deaths Contribution by Health Facility Level (Jan-June, 2025)

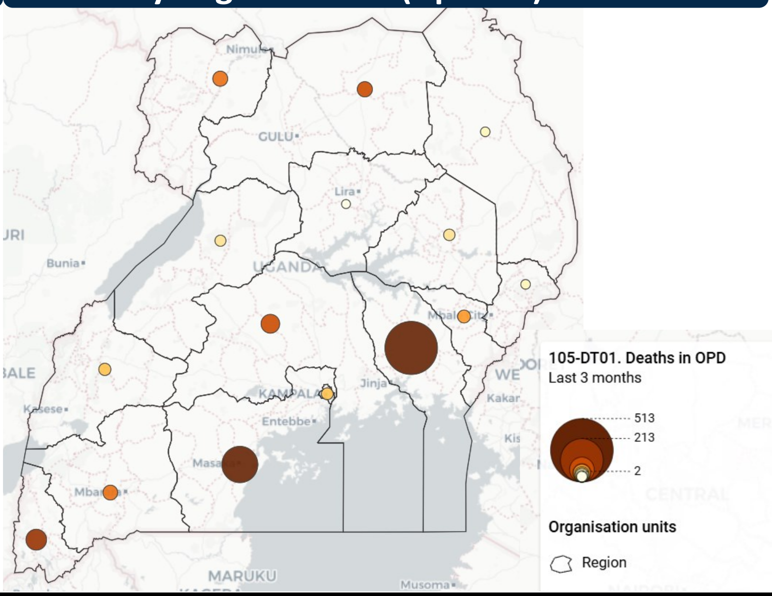
For the period under review, majority of health facility deaths (73.1%) occurred within the IPD with the highest contributor being the General Hospitals (GH) standing at a 30%. These were followed closely by Regional Referral Hospitals (RRHs) at over 25%, HCIVs at 20% and National Referral Hospitals (NRHs) at 12%.

It should be noted that more than 95% of OPD deaths were reported from levels below the HCIVs

Deaths at IPD by Sex and Age Gp



Deaths by Regional level (Apr-Jun)



Mortality data from national & Regional Referral Hospitals (Jan-June, 2025)

There are inconsistencies in OPD death reporting among the national and Regional Referral Hospitals (RRHs). In January, only Arua and Mulago reported deaths, while no data were submitted by Uganda Cancer Institute & Uganda Heart Institute.

Table 1: Mortality data from national and Regional Referral Hospitals (OPD and IPD)

	Jan		Feb		Mar		Apr		May		Jun	
RRHs	OPD	IPD	OPD	IPD	OPD	IPD	OPD	IPD	OPD	IPD	OPD	IPD
Arua	16	61		77		71		10		12		10
Butabika		4		8		7		3		11		
China Uganda / Naguru		35		38		38		19		33		39
Entebbe		33		21		23		40		35	0	43
Fort Portal		108		72		73		74		76		96
Gulu		51		68		11		13		39	0	39
Hoima	0	84	0	86		74	0	63		74		61
Jinja		154		98		152		137		124		97
Kabale	0	36	0	33	0	39	0	30	0	57	0	52
Kawempe National		108		122		119		120		86		118
Kayunga		68	1	59		69		58	5	65		68
Kiruddu National		145		112		137		129		128		109
Lira		146		129		132		93		112		105
Masaka		143		122		112	0	144		141	0	114
Mbale	0	231	0	165		189		166		151	0	160
Mbarara		100		110		157		179		195		198
Moroto		40	0	42	0	54	0	47	0	42	0	39
Mubende	0	93		60		72	30	74		70		72
Mulago National	17	858	0	645	0	747	0	695	0	786	0	614
Soroti		91		94		72		85		82		90
Uganda Cancer Institute												
Uganda Heart Institute												
Yumbe	0	21		13		17		18	0	14	0	25

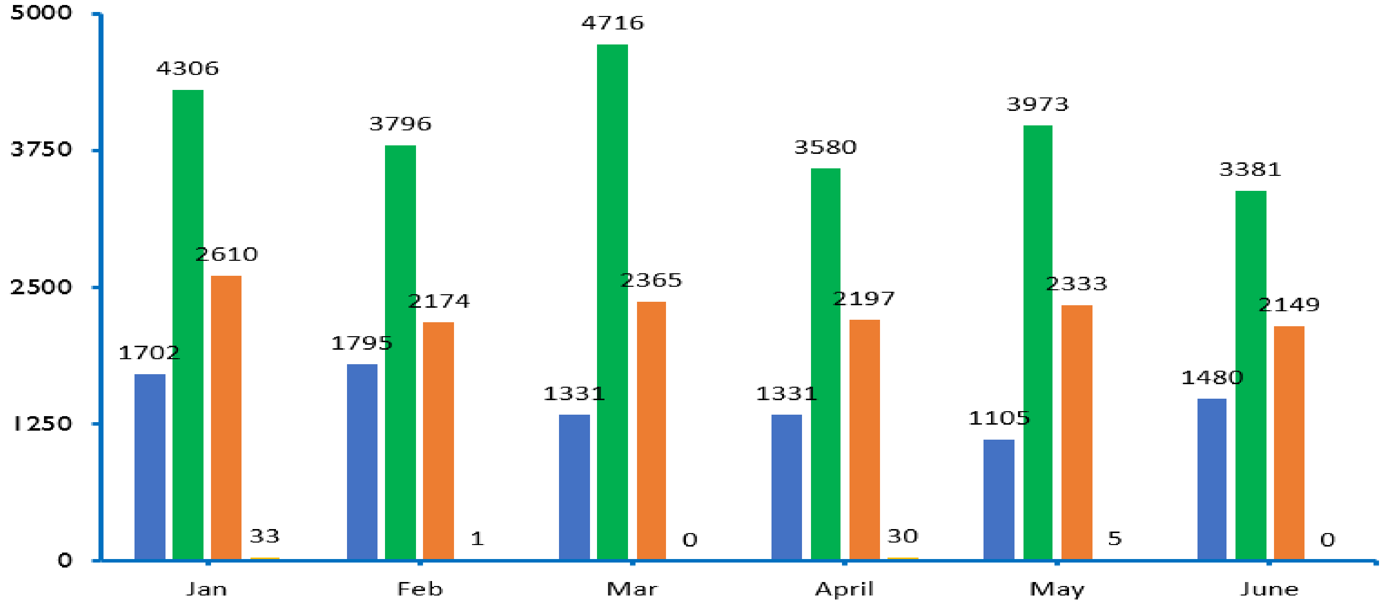


Figure 4: Contribution of RRHs to OPD & IPD deaths.
Most facility-level deaths occurred within the IPD and RRHs contributed over 50% of all monthly IPD deaths. More than 95% of OPD deaths were reported from levels below the RRHs

Maternal and Perinatal Deaths at Health Facility

Between **Jan to June 2025**, Uganda registered a total of **467** Maternal Deaths, and **3411** Newborn Deaths with a peak in the month of April. However, despite facilities ensuring all maternal and perinatal deaths are reported, we face a huge disparity in death audits. The number of maternal and perinatal deaths reviewed is far lower than those reported.

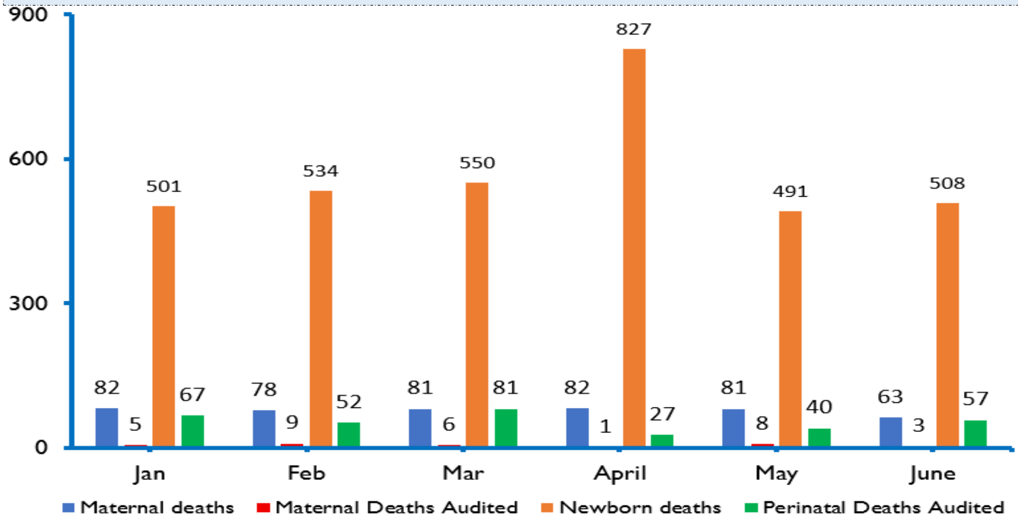


Figure 5: Maternal and perinatal deaths reported compared to those audited.

For example, in January, only **5** out of **82** maternal deaths were audited, and this gap remains consistent across the months.

Similar findings are observed for Newborn deaths. Out of the **501** newborn deaths reported in January, only **57** were audited (*Figure 7*).

Deaths Notification and Medical Certification at Health Facility

CHALLENGE:

Out of the total deaths reported, the proportion notified remains below 50% for each reporting month, while the proportion medically certified is even lower (under 40%).

Considering the trend along the target period, Uganda is experiencing a worrying decline in medical certification of cause of death. The data shows a decline in both indicators, highlighting a major gap in determining the leading cause of mortality in the Country.

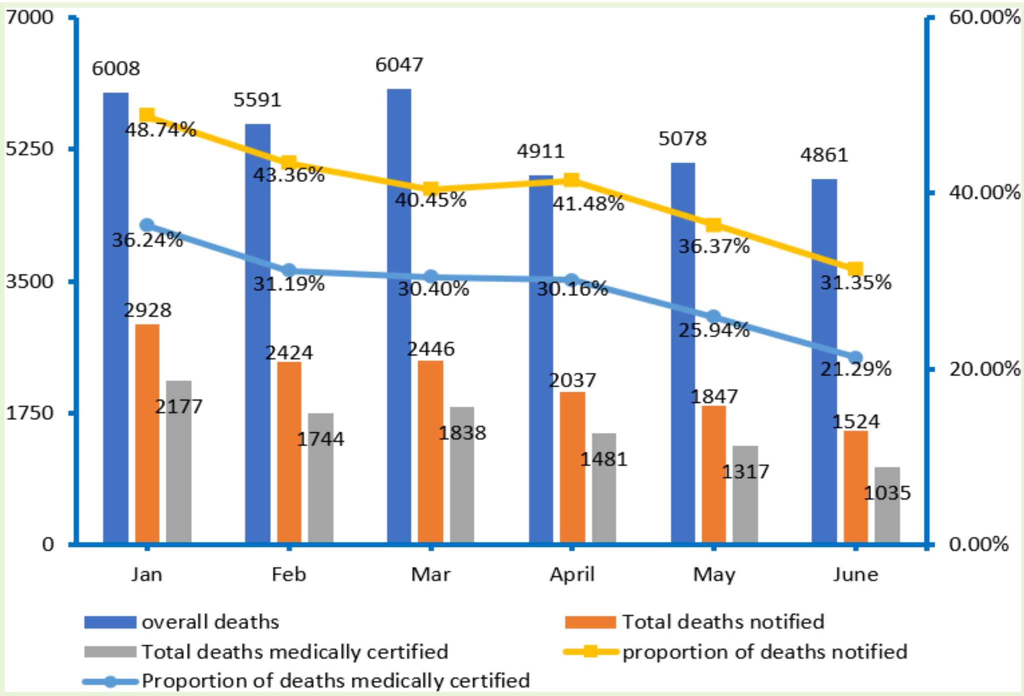


Figure 6: Proportion of HF deaths reported, notified, & medically certified.

NOTE: Incomplete death **identification** and **notification** limits reviews, **cause-of-death determination**, and data sharing, leaving communities and the Ministry of Health without a clear picture of what is killing people. To close this gap, we must ensure complete death reporting at both community and Health facility level, investment in systems and capacity for death reviews and cause-of-death assignment.

Without an efficient mortality surveillance mechanism , public health surveillance and response will be hampered

10 Leading causes of death reported for the month of June 2025

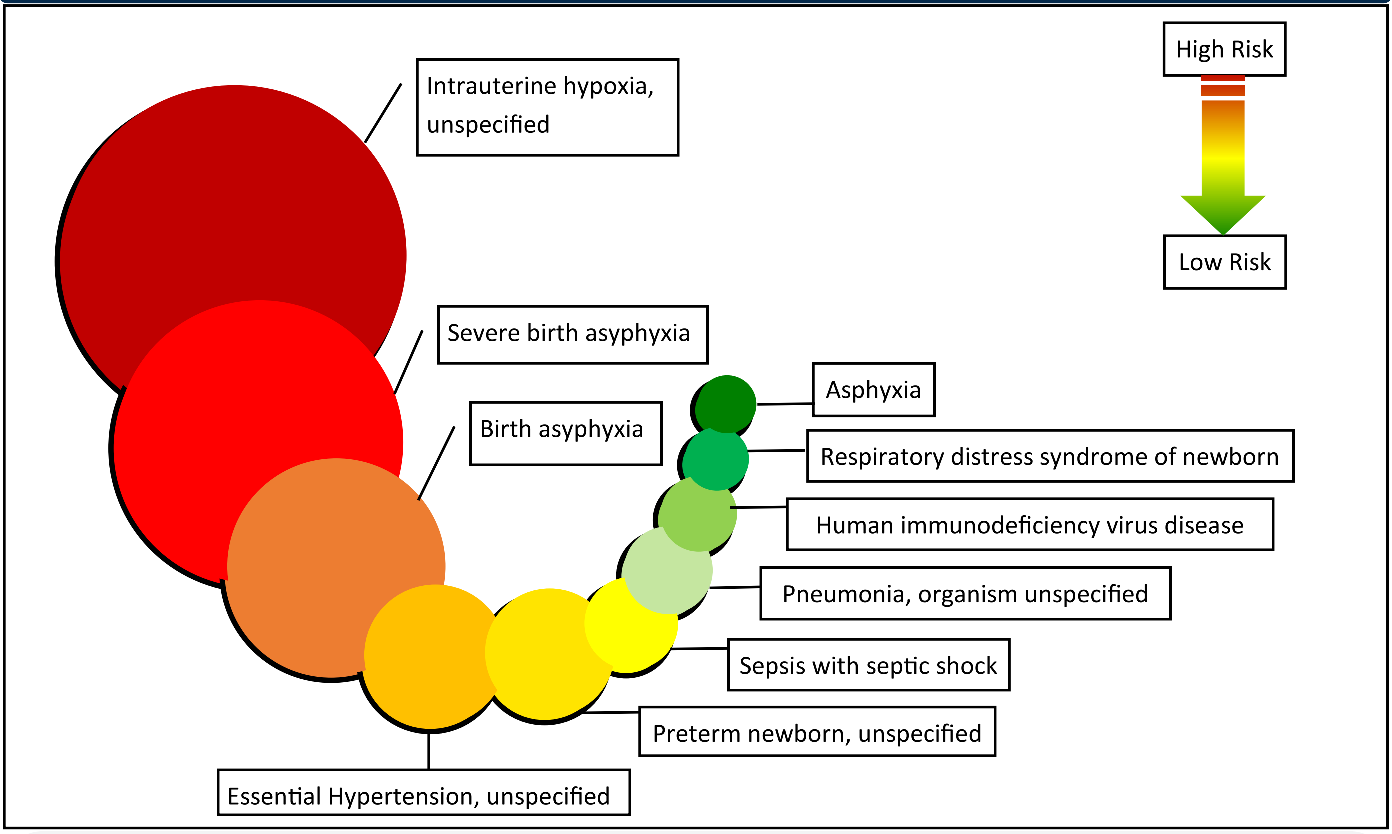


Figure 7: Top 10 causes of death among the Medically certified. The leading causes of death reported through HMIS 100 show recurring patterns across the months. Consistently reported causes include birth asphyxia, pneumonia (unspecified organism), intrauterine hypoxia, severe birth asphyxia, preterm newborn (unspecified), and essential hypertension. HIV clinical stage IV appeared among the top 10 causes in January, March, and April. In addition, noncommunicable diseases such as stroke, diabetes, and essential hypertension were also among the leading causes throughout the period.

HIV related deaths and their causes (HMIS 106a)

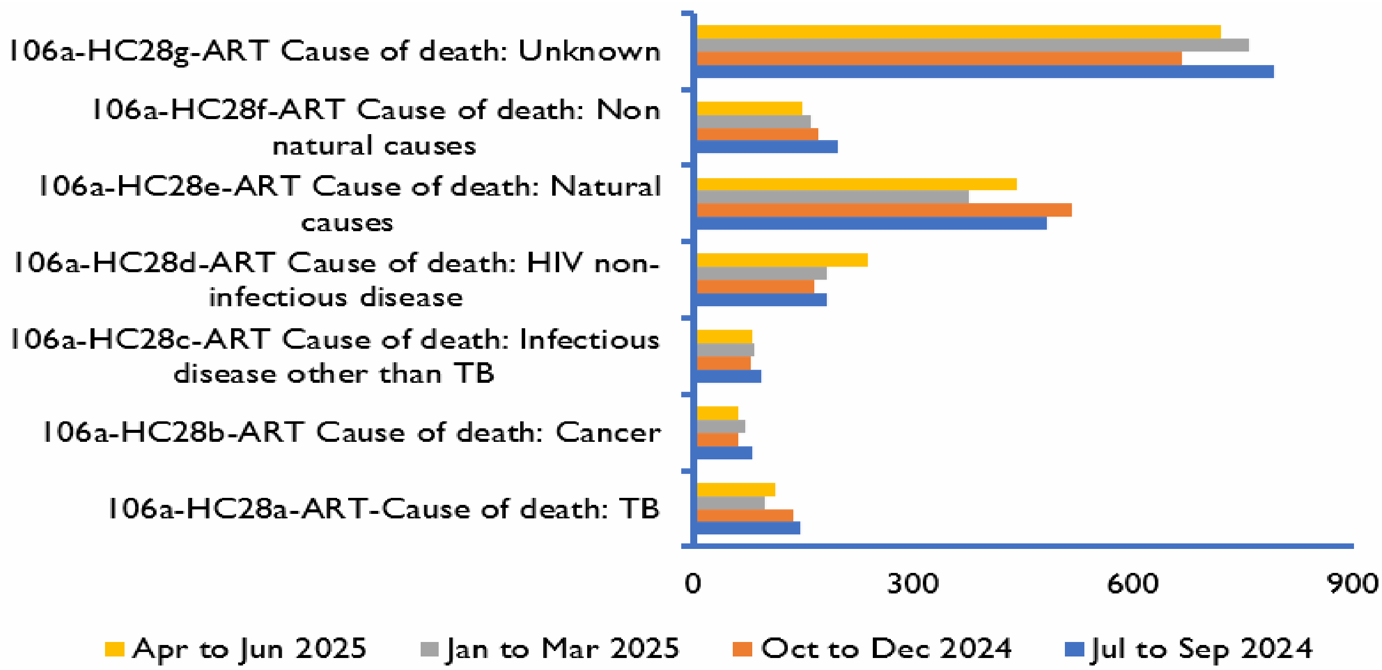
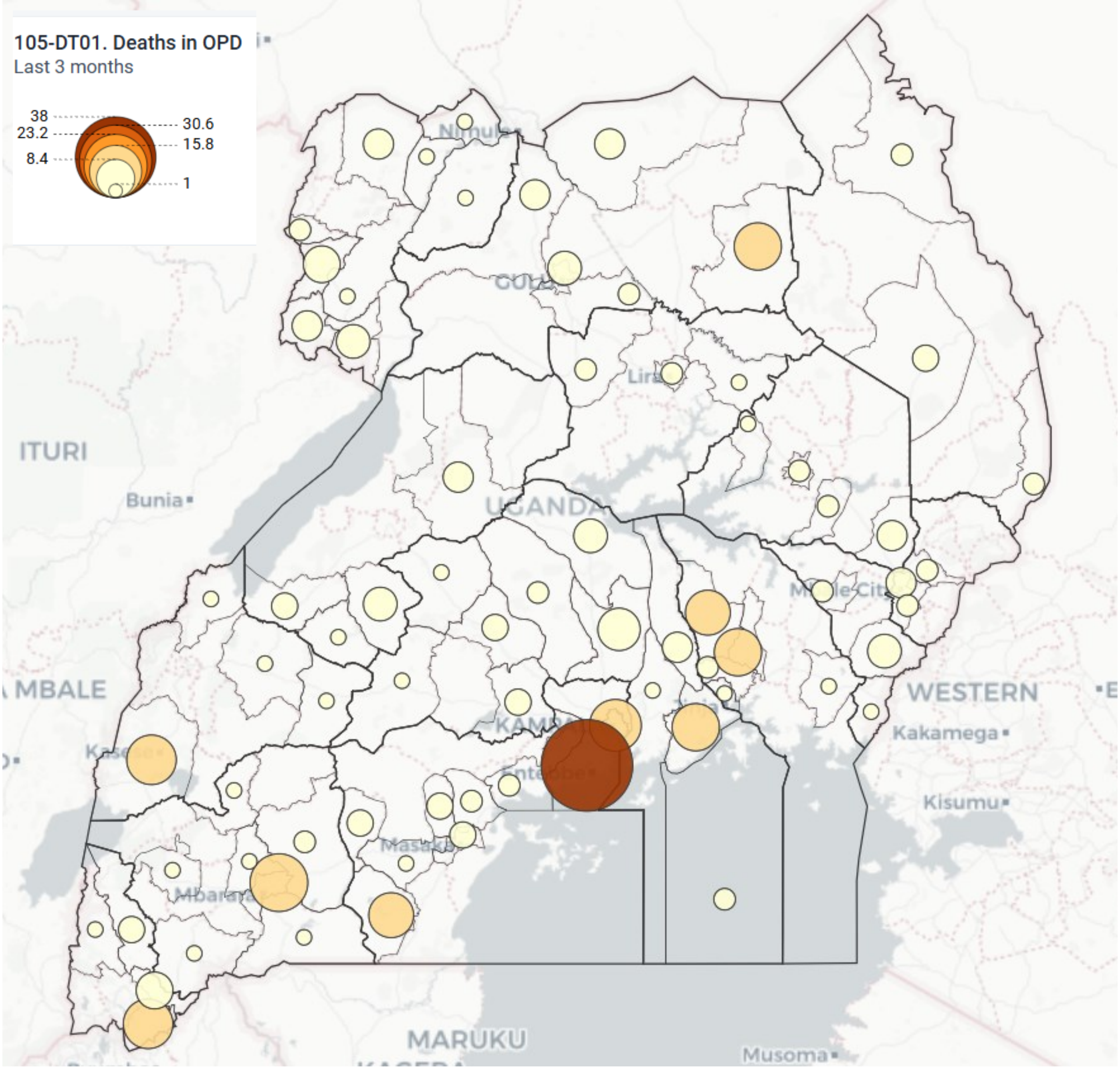


Figure 8: Causes of death among HIV Patients with no clinical contact since last expected contact confirmed dead after tracing by cause of death.

Distribution of deaths by district for April to June 2025



For the last Quarter, the Moh has registered an improvement in coverage of death reporting from 57% to 68%. This has been greatly contributed to by 10 districts of Wakiso—38, Mbarara—15, Kampala—12, Kasese—11, Kabale - 11, Buikwe—10, Luuka—10, Agago—10, Rakai—9, and, Kamuli—9.

Nevertheless, we have realized a vacuum of reporting in the Karamoja and Albertine regions, a call for action

Mortality Surveillance implication and call for action

Understanding who dies, where and from what causes is central to improving public health and responding to emerging threats. Death notification and reviews help in understanding circumstances surrounding the death and inform quality improvement/systems strengthening efforts, while assignment of cause of death help to appreciate burden of disease hence informing resource allocation.

Compiled by: Caroline Kyozira, Bulamu Martin, Benard Owori, Mark Outeke .
Oversight: Dr. Allan Muruta , Commissioner – Integrated Epidemiology, Surveillance & Public Health Emergencies